

MEDICAL DISCHARGE SUMMARY

Patient Name: John Smith

Age/Sex: 58-year-old Male

Hospital Number: MRN-458721

Admission Date: 15 July 2026

Discharge Date: 18 July 2026

Chief Complaint

Chest pain radiating to the left arm for approximately two hours, associated with sweating and mild shortness of breath.

History of Present Illness

The patient presented to the emergency department after experiencing sudden onset central chest pain while walking. The pain was described as heavy, 8/10 in intensity, radiating to the left shoulder and arm. Past medical history included hypertension, type 2 diabetes mellitus, and hyperlipidemia. The patient reported smoking one pack of cigarettes daily for over 25 years.

Physical Examination

Blood Pressure: 156/94 mmHg

Heart Rate: 102 bpm

Respiratory Rate: 20 breaths/min

Temperature: 36.8°C

Oxygen Saturation: 97% on room air.

Investigations

ECG demonstrated ST-segment elevation in leads II, III, and aVF, consistent with an acute inferior ST-elevation myocardial infarction (STEMI). Troponin-I was markedly elevated at 12.6 ng/mL. Complete blood count and renal function were within normal limits. Random blood glucose measured 198 mg/dL. Lipid profile revealed elevated total cholesterol (238 mg/dL), LDL cholesterol (162 mg/dL), HDL cholesterol (39 mg/dL), and triglycerides (214 mg/dL).

Hospital Course

Emergency coronary angiography revealed a 95% occlusion of the right coronary artery. Successful percutaneous coronary intervention (PCI) with placement of a drug-eluting stent was performed without complications. The patient remained

hemodynamically stable and participated in early cardiac rehabilitation.

Discharge Medications

Aspirin 81 mg daily, Clopidogrel 75 mg daily, Atorvastatin 80 mg nightly, Metoprolol Succinate 50 mg daily, Ramipril 5 mg daily, and Metformin 500 mg twice daily.

Discharge Advice

Continue all prescribed medications, follow a low-salt and low-fat diet, stop smoking completely, gradually increase physical activity as tolerated, attend cardiac rehabilitation, and follow up with the cardiology clinic in two weeks. Seek immediate medical attention if chest pain, severe shortness of breath, or fainting recurs.

This is a fictional medical report created for educational and software testing purposes. It is not based on a real patient.